



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1087/12

BEFORE:

S. Clement : Vice-Chair
B. M. Young : Member Representative of Employers
R. J. Lebert : Member Representative of Workers

HEARING:

May 24, 2012 at Windsor
Oral

DATE OF DECISION:

July 30, 2012

NEUTRAL CITATION:

2012 ONWSIAT 1713

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer (ARO) dated
April 8, 2010

APPEARANCES:

For the worker:

Did not participate

For the employer:

T. Bergeron Lucha, Lawyer

REASONS

(i) Introduction to the appeal proceedings

- [1] The employer appeals the April 8, 2010 decision of B. Maier-Pessoa, Appeals Resolution Officer (ARO), which concluded that the worker had entitlement for a recurrence of the August 15, 2006 left knee injury on March 5, 2008 and found the worker was entitled to loss of earnings (LOE) benefits for the period of March 5, 2008 to October 19, 2008. The ARO rendered a decision based upon the written record without an oral hearing.

(ii) Issues

- [2] The issues under appeal are as follows:
1. Did the March 2, 2008 left knee injury represent a recurrence of the August 15, 2006 injury?
 2. Was the worker entitled to LOE benefits from March 5, 2008 to October 19, 2008?

- [3] The appeal is denied for the reasons set out below.

(iii) Background

- [4] The following are the basic facts.

- [5] The worker started employed with the accident employer as an industrial mechanic and began his employment in 2000. The worker was injured on August 15, 2006 when he stepped on a piece of angle iron and slipped off and hyper-extended his left knee.

- [6] The worker was seen initially seen for first aid at work and had two further visits at the first aid office at work before being seen by his doctor in October 2006. The diagnosis was left knee strain.

- [7] The worker was referred to an orthopaedic specialist who confirmed the diagnosis of left knee sprain. An MRI examination did not reveal a tear. The worker was prescribed a brace and was referred for physiotherapy for treatment of the left knee instability. Initially the worker was granted three days of LOE benefits. Following an objection by the employer the LOE benefit was reduced to one day.

- [8] On March 3, 2008, the worker claimed he sustained a recurrence of the left knee injury when he slipped and fell at home. The Board granted entitlement for a recurrence and paid LOE benefits from March 5, 2008 to October 19, 2008.

- [9] The employer objected to the Board decision to grant entitlement for a recurrence and to the payment of LOE benefits. The employer's appeal was considered by the ARO who upheld the decision to grant entitlement for the recurrence and to pay LOE benefits. The employer appealed and that issue is now before us.

(iv) Law and policy

[10] Since the worker was injured in 2006, the *Workplace Safety and Insurance Act, 1997* (the “WSIA”) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[11] Specifically, section 43 of the WSIA governs the worker’s entitlement in this case.

[12] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, Revision #8, would apply to the subject matter of this appeal:

- Package #31 – Secondary or Non-Work-Related Conditions
- Package #36 – LOE Benefits
- Package #300 – Decision Making/Benefit of Doubt/Merits and Justice

[13] We have considered these policies as necessary in deciding the issues in this appeal, in particular:

- *Operational Policy Manual (OPM) Document #15-03-01, Recurrences*
- *OPM Document #11-01-13, Benefit of Doubt*
- *OPM Document #18-03-02, Payment of LOE Benefits*

(v) Testimony

[14] The Panel heard the testimony of Mr. R. L., a co-worker. The witness stated he had been employed as a Millwright with the employer since 1993. He explained he repairs machines through the plant, including repairs in the body shop which is where he was working in August 2006.

[15] In response to questions the witness acknowledged he knew the worker. He said that he would only have worked with the worker once because of shift rotations. He confirmed he knew the worker outside of work because he had coached the worker’s son in hockey. He said other than seeing the worker at the arena he did not see the worker outside of work. He said he was aware that the worker would have played in the skilled trades’ golf tournament but he was not partnered with him so he couldn’t offer any information other than the knowledge that the worker was in the tournament.

[16] The witness did recall the worker having lost time from work due to a problem with his knee but said his recollection of this was vague.

[17] The witness said he did recall the worker complaining about an injury to his knee saying he had “popped” his knee. The witness said he could not recall when that was or how it happened. The witness stated work is done while standing and walking on a screen guard which is flexible. He said although he had not hurt himself he could see how the worker might have hurt himself. He said working while on the screen guard is difficult in that it is flexible and is difficult to stand and walk on. The witness could not recall how long after the initial complaint that the worker complained again.

[18] The witness confirmed he had been called by a Board investigator and he told her what he had heard, specifically that he had heard the worker yell and the witness told the investigator he thought the worker had said he had “popped” his knee.

[19] The witness was unable to offer any other information.

(vi) Submissions

[20] Ms. Bergeron Lucha made oral submissions on behalf of the employer. She stated that it is the employer's position that applicable board policy requires continuity and in this case continuity is not evident. She pointed out that the witness's recollection of what happened and the worker's description of the August 2006 accident differ. She pointed out that the witness was unable to provide evidence regarding the specifics of the 2006 accident.

[21] She submitted that entitlement for a recurrence requires a link between the original accident and the knee problems in 2008 and it is the employer's contention that a link cannot be established in the case before us. She pointed out there was a lot of time between the 2008 claimed recurrence and the Board investigation in 2009 and the Board erred in placing weight on the results of that investigation. She reiterated that there is disconnect between the original injury and the 2008 recurrence of symptoms. She contends the March 2008 slip and fall accident at home was responsible for the worker's subsequent knee symptoms and meniscus tear.

[22] Ms. Bergeron Lucha submitted that there must be an existing impairment to cause a recurrence or a second accident. She pointed out that in 2006 the injury was diagnosed as a knee strain and this diagnosis was not made until two months after the accident. The description of the accident was a hyperextension yet the diagnosis was a strain. She noted that in December 2006 Dr. Anisette, an orthopaedic surgeon, suggested a knee brace but Dr. Yavanovich, Independent Medical Assessor, didn't understand why a brace was prescribed when there was no tear. Ms. Bergeron Lucha asked the Panel to consider the results of the MRI which did not demonstrate any tears. She said although the worker made some visits to the first aid department at work the worker did not seek any other medical attention. It was her submission that it cannot be shown that the visits to first aid had any relationship to the August 2006 injury. She submits the medical evidence shows degenerative changes.

[23] Ms. Bergeron Lucha asked the Panel to consider material taken from an internet website. That article from Medical Disability Guidelines entitled Sprains and Strains, Knee is one which distinguishes differences between knee strains. She noted a first degree strain stretches a ligament or muscle but does not cause structural damage. She said this type of strain is characterized by edema or swelling, localized pain and tenderness, and no marked instability. With this type of strain, she said an individual is capable of working within the limits of the pain without causing further damage to the knee. Treatment consists of NSAIDs, ice, and rest. She said the literature indicates healing time is usually 2 to 6 weeks with full recovery expected.

[24] She pointed out a second degree strain/sprain partially disrupts or weakens the ligaments or muscle tendon unit and is characterized by a partial tear, moderately localized pain and tenderness and mild instability. With a second-degree strain, the injured structure should be protected for about six weeks to allow for prevention of further injury. Treatment includes NSAIDs, ice, rest and physical therapy. She stated the literature states the healing time is typically six weeks with full recovery expected, especially with physiotherapy.

[25] Ms. Bergeron Lucha noted the literature states a third degree strain/sprain is one where the ligament or muscle tendon is completely disrupted or torn and requires a long period of protection and often surgical repair.

- [26] Ms. Bergeron Lucha reiterated that it is the employer's position that the March 2008 symptoms are attributable to the slip and fall at home and not the 2006 workplace injury. She submitted the treatment following the 2006 injury which consisted of ice and aspirin was for degenerative changes and not a workplace injury. She pointed out the worker was able to continue to do his regular work with no restrictions and didn't require any type of regular treatment. She submitted that the worker's 2006 injury would have resolved prior to the 2008 accident the worker had at home. She reiterated the employer's view that it is more probable than not that the worker's symptoms and the meniscal tear in March 2008 was attributable to the accident the worker had at home. Alternatively the meniscal tear may be attributable to the degenerative changes in the worker's knee. It remains the employer's position that neither of these causes can be attributable to the 2006 workplace accident.
- [27] Ms. Bergeron Lucha submitted the criteria for entitlement for a recurrence, as spelled out in Board OPM Document #15-03-01, *Recurrences* was not met. It was her submission that there was no compatibility between the knee injury of August 2006 and the knee injury of March 2008. She pointed out that even though the worker required physiotherapy following the 2006 accident the worker was still able to perform his regular duties following the injury. She stated this was not the case following the 2008 slip and fall at home. She pointed out that following the slip and fall at home that this changed. She said it is the employer's position that the second injury was a significant one and was responsible for the diagnosed meniscus tear. She pointed out the policy on recurrences does not define the benefits which would flow from a recurrence.
- [28] Ms. Bergeron Lucha submitted the criteria spelled out in Board OPM Document #18-03-02, *Payment of LOE Benefits* were not met. She stated that in order to receive LOE benefits it must be shown that the loss of earnings is attributable to the work-related injury. She submitted in the case before us that the loss of earnings from March 2008 to October 19, 2008 was not attributable to the workplace injury but rather was the result of the injury sustained in the slip and fall the worker had at home. She asked the Panel to consider the weather conditions on the night of that fall and it was her submission that the weather conditions were such that the ground would have been icy and covered by some snow which would lend itself to a significant fall.
- [29] Ms. Bergeron Lucha submitted that the ARO erred in placing too much emphasis on the report of the Board's investigator and the opinion of the Board's Medical Consultant. She submitted the Board Medical Consultant's opinion was impacted by the bias in the questions posed by the claims adjudicator in the memo dated June 9, 2009.
- [30] Ms. Bergeron Lucha asked the Panel to consider the analysis in *Decision No. 1445/09* and apply this to the case before us. In that decision the Panel found the evidence failed to support clinical compatibility between the worker's accident history and the need for surgical treatment in 2007.
- [31] On behalf of the employer Ms. Bergeron Lucha asked that the decision to grant entitlement for the recurrence and to pay LOE for the period of March 5, 2008 to October 19, 2008 be rescinded. The employer is asking for a full refund of the costs of LOE in the form of a credit to the employer's account and a retroactive adjustment on the employer's New Experimental Experience Rating (NEER) statement.

(vii) Analysis

[32] The question which needs to be considered by the Panel is whether or not the worker had a recurrence in March 2008 and if so is the loss of earnings for the period of March 5, 2008 to October 19, 2008 attributable to the August 2006 workplace accident.

[33] Board OPM Document #15-02-01, *Recurrences* sets out the factors to be considered in determining whether or not a worker had a recurrence and states:

Policy

A worker is entitled to benefits for a recurrence of a work-related injury or disease.

A recurrence may result from an insignificant new accident, or may arise when there is no new accident. To identify a recurrence, the WSIB must confirm that there is clinical compatibility between the original injury or disease and the current condition, or a combination of clinical compatibility and continuity.

If a significant new work-related accident occurs, the WSIB establishes a new claim.

Guidelines**Recognizing a recurrence****Clinical compatibility**

To establish clinical compatibility, a decision-maker compares the worker's current clinical condition to that following the initial accident. The decision-maker considers

- whether the parts of the body affected now are the same as, or related to, those affected initially
- whether the body functions affected now are the same as those affected initially, and
- the degree to which body functions are affected now (as compared to the effect of the initial condition).

Similar clinical conditions indicate that the current problem or problems may be a result of the original injury, whereas dissimilar or unrelated clinical conditions indicate that there is no compatibility, and therefore no recurrence.

Continuity

To establish continuity (i.e., a connection between the original clinical condition and the most recent problem or problems), the decision-maker considers whether the worker has

- complained to supervisors, co-workers, or health care practitioners on an ongoing basis since the original injury
- demonstrated ongoing symptoms since the original injury
- required work restrictions or job modifications

had ongoing treatment for the original condition, or

experienced a lifestyle change since the original accident (e.g., has the worker become unable to participate in household duties, or social or recreational activities?).

In complex cases, the decision-maker may consult with WSIB medical staff before making a decision.

[34] The Panel has considered the evidence in the case materials in conjunction with the requirements of Board OPM Document #15-03-01, *Recurrences*. The worker experienced an onset of left knee pain on August 15, 2006 when he stepped on a piece of angle iron and slipped

off and hyper-extended his left knee. The worker reported the accident to the employer and initial treatment was provided through the plant's first aid department. The worker sought further treatment in October 2006 and was seen by his family physician, Dr. Albus. In the Health Professional Report dated October 10, 2006, Dr. Albus reported the accident as a hyperextension of the left knee while pushing a van. The diagnosis in October 2006 was a knee sprain. Dr. Albus identified restrictions on standing, kneeling and climbing stairs and ladders for more than 14 days. He indicated he would see the worker in follow-up.

- [35] The worker was referred to Dr. Annisette, orthopaedic surgeon. In a letter to Dr. Albus dated December 12, 2006, the orthopaedic surgeon identified the injury as a hyperextension twisting injury to the left knee. The pain was identified as posterior pain associated with difficulty on extension and a feeling of instability. The physical findings included marked posterior knee tenderness with the knee going into some hyperextension with pain. Dr. Annisette stated there were no ACL findings; mild positive Sach's sign and no joint line tenderness. The clinical impression was PCL tear/insufficiency of the left knee. The plan of care included an MRI, PCL bracing and physiotherapy.
- [36] The worker was referred for physiotherapy and based on the records contained in the case materials the worker had 22 physiotherapy treatments between December 13, 2006 and March 20, 2007.
- [37] The worker had an MRI on January 8, 2007. In the report of that exam the findings were identified as a very minimal inter-substance degeneration but no tear with the surround structures being normal.
- [38] In a follow-up letter dated January 30, 2007 Dr. Annisette stated the worker is better with his brace. He noted the MRI showed an intact ACL/PCL. He said the brace controlled the hyperextension.
- [39] Following the completion of the physiotherapy and a return to his regular job, the worker reported he had recurrent symptoms which would last two to three days. He told the Board Claims Investigator that when he experienced recurrent symptoms he would report to First Aid and at home would rest his knee, apply ice and take medication including Tylenol #3 and Ibuprofen. He thinks he may have told Dr. Albus about the ongoing symptoms when he was seen for other reasons. The worker told the Claims Investigator that he was learning to live with the ongoing occasional symptoms and did not seek formal medical attention.
- [40] Contained in the case materials are records of the worker's visits to the First Aid Department. In 2006 the worker was seen on a number of occasions with complaints of left knee pain, specifically on October 11; October 30; November 21, 2006. These visits were in addition to the medical treatment he received from his family physician and the orthopaedic surgeon. In 2007 in addition to the physiotherapy the worker sought attention for knee pain from the First Aid department on January 22, 2007; February 22, 2007; March 8, 2007; April 14, 2007; August 29, 2007. The Panel had regard to the worker's report to the Occupational Health Nurse in first aid that the onset and/or worsening of left knee pain was associated with the physical demands of the work he was performing. It is clear from the first aid notes that the physical demands of the work served to exacerbate the left knee condition and bring on left knee pain. This evidence is consistent with the statement the worker provided to the Board's Claims Investigator.

- [41] The Panel had regard to the oral testimony of the witness as well as what the witness told the Board Claims Investigator. The witness was aware of the worker's occasional ongoing complaint of left knee pain. The witness told the Claims Investigator that working in the basket/cages seemed to aggravate the knee more whereas during his testimony the witness explained that working while on the screen guard is difficult because it is flexible and is difficult to stand and walk on. During his testimony the witness had difficulty recalling any other information. The Panel notes the hearing was held six years after the compensable accident and therefore it is not unreasonable to have a witness who cannot recall details of something that occurred years earlier.
- [42] On March 2, 2008, the worker slipped and fell while taking the trash out. In the report of injury on April 2, 2008, the worker said while taking out the garbage his knee gave out and he slipped and fell on the left knee and twisted it. In his statement to the investigator on January 2, 2008, the worker said he went outside with the trash in his hand and while in the process of walking the left knee hyper-extended as it had on previous occasions. He said he was unable to brace himself as he had on previous occasions and his opposite leg gave out when he put additional force on it and he fell twisting and falling on the left knee. He reported he experienced an immediate onset of more severe left knee pain. He saw his family physician and took the first available appointment. He was referred to a specialist again and returned for physiotherapy. The question this raises is whether or not this incident would serve to break the chain of causation or whether the worker experienced a recurrence.
- [43] Board OPM Document #15-03-01, *Recurrences* states that a recurrence may result from an insignificant new accident or may arise when there is no new accident. Given this criteria the Panel had regard to the medical reporting following the March 2, 2008 slip and fall. In the continuity report dated March 25, 2008 Dr. Albus stated the worker had past knee pain with undiagnosed instability. He noted the worker had increased pain and mild effusion and queried the possibility of an internal derangement. Dr. Albus reported he saw the worker on March 4, 5, 10 and 25, 2008.
- [44] The worker was referred to Dr. Annisette and in a letter dated May 6, 2008 Dr. Annisette reported the worker had continued to experience problems with the knee hyper-extending and giving out with the most recent incident occurring in March 2008. Dr. Annisette arranged for an MRI. The result of the MRI demonstrated the worker had a high grade partial thickness tear of the PCI with no evidence of an ACL tear and no meniscal tear.
- [45] The Panel gives significant weight to the opinion and findings of Dr. Annisette. When reviewing Dr. Annisette's findings in his letter of January 30, 2007 and comparing it to his findings following the March 2008 slip consistency exists in that the worker continued to experience problems with hyperextension of the left knee. Dr. Annisette prescribed a knee brace for the purpose of controlling the hyperextension. Despite having the brace the worker did continue to experience problems with the left knee and in fact the worker reported his knee hyper-extended on March 2, 2008 as it had on previous occasions but he was unable to brace himself with the opposite leg as he had done previously because he lost his footing on the ice and he slipped and fell to the ground. The fact that the worker required a brace to stabilize the left knee following the August 2006 accident is evidence of continuity.
- [46] The worker was seen by Dr. Yovanovich, orthopaedic surgeon for an independent medical evaluation in June 2008. In his opinion, Dr. Yovanovich was unable to rule out the

possibility that the worker may have sustained other internal derangements such as a meniscal tear in the absence of an MRI examination. When asked, Dr. Yovanovich was unable to identify the reasons a custom brace was recommended.

[47] In a memo dated June 25, 2009, Dr. Lau, a Board Medical Consultant responded to a query by the claims adjudicator. In that memo, Dr. Lau stated he had reviewed the medical information on file. Based on a review of the medical information Dr. Lau concluded the partial thickness tear and hyperextension injury of the left knee was compatible with the August 15, 2006 injury. Dr. Lau also stated the continuation of the problem from 2006 to March 2008 was anticipated as described by the worker with the recurrence of the hyperextension injury. He concluded the recurrence of March 3, 2008 was compatible with the history of the August 15, 2006 accident. The Panel does not accept the contention of Ms. Bergeron Lucha that Dr. Lau's opinion was biased by the nature of the claims adjudicator's request for a medical opinion. Dr. Lau had full opportunity to review the medical evidence and reach an independent conclusion.

[48] The Panel has considered the medical opinions and we give significant weight to the opinion of Dr. Annisette who prescribed the use of a knee brace which Dr. Annisette identified as being useful to control the hyperextension. We also had regard to Dr. Annisette's comments that the worker continued to have problems with the left knee including hyperextension and giving out. This in fact is what the worker described as having happened in March 2008.

[49] Based on all the preceding the Panel is satisfied that there is both continuity of complaint and compatibility as required under Board OPM Document #15-03-01, *Recurrences*. The evidence demonstrates the same part of the body is affected, specifically the left knee with recurring pain and hyperextension. The worker did complain to and sought treatment at first aid. As well he complained to a co-worker and he had ongoing symptoms of pain and hyperextension of the left knee. The worker was prescribed a custom left knee brace for treatment of the left knee injury following the August 15, 2006 accident which Dr. Annisette indicated served to control the hyperextension.

[50] The Panel acknowledges that the evidence demonstrates the weather at the time of the March 2008 slip and fall made a contribution to the worker's knee injury. We also have the worker's statement that the hyperextension and giving way of his left knee caused the fall. The provisions of OPM Document #11-01-13, *Benefit of Doubt* stipulates the following:

Law

The *Workplace Safety and Insurance Act* provides:

s. 119(2)

If, in connection with a claim for benefits under the insurance plan, it is not practicable to decide an issue because the evidence for and against it is approximately equal in weight, the issue shall be resolved in favour of the person claiming benefits.

Policy

In determining any claim under the Act, the decision shall be made in accordance with the real merits and justice of the case, see 11-01-03, *Merits and Justice*. Where it is not practicable to determine an issue because the evidence for or against the issue is approximately equal in weight, the issue shall be resolved in favour of the person claiming benefits.

Guidelines

This policy is not to be used as a substitute for evidence. It is applied when the facts of a case are so evenly balanced that a clear decision is impossible.

- [51] The Panel finds the evidence regarding the cause of the March 2008 accident at home to be more than equal in weight. What is clear to the Panel is that there exists continuity of complaint and compatibility with the August 2006 left knee injury and that injury made a significant contribution to the left knee problems from March 2, 2008 onward. The worker is entitled to a recurrence.
- [52] In reaching our decision the Panel did have regard to the representative's reference to *Decision No. 1445/09*. In that decision the Panel took note of the fact that the worker's treating physicians did not provide or explain how the worker's osteoarthritic changes could be related to the worker's injury. The Panel also took note of the opinion of another doctor who concluded the workplace injury made a pre-existing condition symptomatic but concluded the accident was not responsible for the meniscus tear. In that decision the Panel concluded that despite the worker's ongoing complaints of pain the medical evidence failed to support compatibility between the compensable injury and the need for knee surgery. In that case the Panel found the evidence demonstrated the presence of significant degenerative changes in the worker's knee which led to the need for surgical treatment. The Panel denied the worker's appeal and found the knee injury was not a significant contributing factor to the worker's ongoing disability. The analysis in that case had regard to the provisions of OPM Document #15-03-01 and the need for clinical compatibility or a combination of clinical compatibility and continuity.
- [53] The facts in the case before us differ. We acknowledge that the evidence, specifically the January 8, 2007 MRI did not reveal any tear, whereas the May 21, 2008 MRI identified a high partial thickness tear of the PCI. While this is a new finding following the March 2, 2008 slip and fall, that fall would not have occurred in the absence of the ongoing hyperextension of the left knee. In a memo dated June 2, 2009 Dr. Lau addresses the differences in the January 8, 2007 MRI of the left knee and the May 21, 2008 MRI of the left knee and concluded that based on the findings in the 2007 MRI which showed minimal inter-substance degeneration, the worker had a mild pre-existing condition which was mildly prolonging and enhancing the worker's recovery. On that basis the employer was granted 50% SIEF relief. In a subsequent memo dated June 25, 2009, Dr. Lau specifically addresses the question of compatibility. Dr. Lau stated the following:
- Medical information on file reviewed. 18 June 2008 IME report noted.
- 21 May 2008 MRI of the left knee indicated high grade partial thickness tear of the PCL. No meniscal tear identified.
- With PCL partial thickness tear, hyper-extension injury of the left knee is compatible with the August 15, 2006 work injury. Continuation of the problem from 2006 to March 3, 2008 anticipated as described by the worker with recurrence of the hyper-extension injury.
- 1) The REO on March 3, 2008 is compatible with the history of the accident of August 15, 2006.
 - 2) Concurred the subsequent lost time and diagnosis from March 3, 2008 is related to the August 15, 2006 work injury.

[54] Having considered all the preceding, the Panel is satisfied that the criteria of compatibility and continuity have been met. The worker has entitlement for a recurrence.

[55] Having found the worker has entitlement for a recurrence, the Panel next considered whether or not the worker is entitled to LOE benefits from March 5, 2008 to October 19, 2008. Section 43(1) of the WSIA provides for the payment of LOE benefits stating:

43(1) A worker who has a loss of earnings as a result of the injury is entitled to payments under this section beginning when the loss of earnings begins. The payments continue until the earliest of,

- (a) the day on which the worker's loss of earnings ceases;
- (b) the day on which the worker reaches 65 years of age, if the worker was less than 63 years of age on the date of the injury;
- (c) two years after the date of the injury, if the worker was 63 years of age or older on the date of the injury;
- (d) the day on which the worker is no longer impaired as a result of the injury. 1997, c. 16, Sched. A, s. 43 (1).

[56] Board OPM Document #18-03-02, *Payment of LOE Benefits* reiterates the provisions of section 43(1) of the WSIA. The policy goes on to provide the guidelines for the payment of full LOE benefits stating:

Guidelines

Payment of full LOE

Treatment with no return to work

If the nature or seriousness of the injury completely prevents a worker from returning to any type of work, the worker is entitled to full LOE benefits, providing the worker co-operates in health care measures as recommended by the attending health care practitioner and approved by the WSIB. If the worker does not co-operate, the WSIB may reduce or suspend the worker's LOE benefits.

[57] The worker cooperated in appropriate health care measures between March 5, 2008 and October 19, 2008, and is therefore entitled to full LOE benefits.

[58] The Panel is satisfied that the Board correctly found the worker had entitlement for a recurrence in March 2008 and granted full LOE benefits from March 5, 2008 to October 19, 2008. The employer's appeal is denied.

DISPOSITION

[59] The appeal is denied.

DATED: July 30, 2012

SIGNED: S. Clement, B. M. Young, R. J. Lebert